

ARCP Evidence Checklist

Quality improvement evidence for AIM trainees — what your portfolio needs, by training year.

THE HEADLINE

- ST4–ST6** QI is **one** route to your annual clinical-governance evidence — alongside audit, Datix review, complaints response or a Coroner's referral. You don't have to lead a full project every year, but keep building a track record.
- ST7** Bring **one** QIP to completion: a full project plan and report, assessed with the Quality Improvement Project Assessment Tool (QIPAT). Starting that project well before ST7 makes this far less last-minute.
- SAM suggests** Raise the game. Be **part of one** QIP as a medical student, **complete one** in Foundation, **one more** in core training and **two** in higher specialty training. That is a track record, not a curriculum rule — the rows above are the rule.

YEAR BY YEAR

STAGE	WHAT'S REQUIRED	TOOLKIT RESOURCE TO USE
ST4	Evidence of engagement with QI, audit, a Datix review, a complaints response, or a Coroner's referral — QI is one acceptable option, not the only one.	Contribute to a ward project; keep your PDSA worksheets and run-chart entries as your evidence.
ST5	Same annual engagement requirement — a good year to move from contributing to co-leading.	PDSA Cycle Worksheet, Run Chart / SPC Worksheet.
ST6	Same requirement — start pitching your own project ahead of ST7.	SMART Aim Statement Builder, QIP Roadmap (Stages 1–3).
ST7	One full QIP: project plan and report, assessed with QIPAT.	Full SQUIRE 2.0 write-up (Reporting Guide + Presentation Template); QIPAT sign-off from your supervisor.

EVIDENCE TO KEEP, MAPPED TO SQUIRE 2.0

EVIDENCE	WHAT IT SHOWS THE PANEL	SQUIRE 2.0 ITEM
SMART aim statement	A clearly defined, measurable problem — not just “things were bad.”	<i>Item 6, Specific Aims</i>
PDSA cycle worksheet, each cycle you run	Iterative testing and real learning from data, not one big-bang change.	<i>Item 13a, Evolution of the Intervention</i>
Run Chart / SPC Worksheet, kept updated	You understand your data over time, not just a before-and-after snapshot.	<i>Item 11, Analysis; Item 13b, Process & Outcome Data</i>
QIPAT form, completed with your supervisor	Formal, supervisor-assessed sign-off — required for your ST7 project.	<i>Curriculum requirement at ST7</i>
Completed SQUIRE 2.0 write-up	A finished, structured report, ready for your portfolio or a presentation.	<i>All 18 items</i>
Presentation or dissemination — local QI meeting, SAM conference, trust newsletter	Evidence you shared what you found, not just that you did the work.	<i>Item 15, Interpretation</i>

THE TIME QUESTION — 30 DAYS A YEAR

The AIM curriculum gives higher specialty trainees one day a week of self-development time on average; the Specialty Advisory Committee settled what that means, and QI is a named use of it.

QUESTION	THE SAC'S ANSWER
How much?	"One day a week, on average" equals 30 full days over 52 weeks . Annual leave, study leave, industrial action and on-call duties do not count against the 30 and do not reduce the total.
Less than full time?	The same 30 days, over an appropriately extended period — pro rata on time, not on days.
Where?	It must be provided, and divided equally between acute internal medicine and specialty-block placements.
What is it for?	Specialist skill and point-of-care ultrasound capabilities, leadership and managerial skills — reflected in the portfolio. If you have not chosen a specialist skill, are not yet ready for POCUS, or have completed one or both, the time is for quality improvement projects , portfolio preparation for supervisor meetings and ARCP, and governance, complaints, leadership, research, education and managerial capabilities.
What must I show?	That the time was used appropriately. Where it is not evidenced, supervisors and TPDs follow the Gold Guide, section 4 ("Progressing as a Doctor") — so log what each day went on, and keep the QI evidence in the table above.
If it can't be scheduled?	Trainee- or site-specific problems go through a governance route: round-table meetings facilitated by the regional education team. Raise it early with your TPD rather than at ARCP.

For a first QIP, that is where the hours come from: the pitch and baseline, a launch week, the weekly counting and diary, and the write-up all sit inside those 30 days — alongside, not instead of, your specialist skill.

RAISE THE GAME — SAM'S SUGGESTED MINIMUM, FROM MEDICAL SCHOOL ONWARDS

The curriculum sets a floor. SAM suggests a track record that builds from the first clinical year, so the ST7 project is the fourth you finish, not the first. Each stage's formal requirement is in its own curriculum; this is the shape to aim for.

STAGE	SAM SUGGESTS	WHAT THAT LOOKS LIKE
Medical student	Be part of one	Join a ward project. Own one measure, one week of counting, or one PDSA cycle — and be named on the write-up.
Foundation	Complete at least one	Your own aim, a baseline, one change, re-measured on a run chart, written up. Small is fine; finished is the point.
Core training (IMT)	One more	This time lead it: a team, two PDSA cycles, a hand-over to whoever comes next.
Higher specialty (ST4–ST7)	Two	One QIPAT-assessed for ST7; one that spreads to another ward or is handed over properly. At least one sent to the SAM library or the SAM conference.

By CCT that is four completed projects and one you were part of — every one of them a run chart, a change and a write-up, and every one of them evidence for the year it was done in.

A FEW PRACTICAL NOTES

- **Protected time is answered above:** 30 self-development days a year, and QI is one of the named uses. Log the days you spend on the project — the time is evidence too.
- **QIPAT is only formally required once**, for your ST7 project — but SAM's own training guidance suggests aiming for roughly one QIPAT-assessed project a year where you can. That's a track record, not a rule.
- **Don't leave it to the week before ARCP.** A write-up takes real time to do properly, and your supervisor needs time to complete the QIPAT assessment with you — start that conversation early.
- **Simply attending QI teaching isn't evidence.** The panel is looking for your own engagement — ideally moving toward a leadership or co-leadership role by ST7.

Sources: AIM 2022 ARCP Decision Aid, JRCPTB / Federation of the Royal Colleges of Physicians of the UK (thefederation.uk); SAM, "Training Information for AIM Higher Specialty Trainees" (acutemedicine.org.uk/training-in-acute-medicine); SAC (AIM) Chair's letter to the SAC team on self-development time, 7 February 2025. The "SAM suggests" ladder is SAM guidance, not a curriculum requirement.